

Risk Mitigation Plan

HealthFirst Care Improvement Initiative

This Risk Mitigation Plan builds directly on the Risk Register and SWOT Analysis produced in the prior lab. It focuses on the seven High-severity risks (Severity = Likelihood × Impact ≥ 6) identified there, and for each one proposes a detailed mitigation strategy and a contingency plan to follow if the risk materializes despite mitigation. The updated Risk Matrix and full contingency plans are also provided in the companion workbook Risk_Matrix_Mitigation.xlsx.

1. High-Priority Risks (from the Risk Register)

Of the 15 risks documented in the prior Risk Register, seven scored High severity (6–9 on the Likelihood × Impact scale) and are the focus of this plan:

- OP-1 — Workflow disruption during phased rollout
- OP-2 — Staff resistance to adopting new workflows
- OP-4 — Resource shortages during go-live
- TE-1 — Data breach / security vulnerability
- TE-2 — System/network downtime
- TE-3 — System integration failure (FR5)
- TE-4 — Legacy data-quality issues misleading decisions

The remaining 8 Medium/Low risks continue to be monitored and reviewed per the cadence defined in the original Risk Management Plan, and are not repeated here.

2. Mitigation Strategies

Mitigation strategies below are specific and actionable, and each is tied to a concrete artifact from the project (a BRD requirement, a WBS milestone, or the Stakeholder Engagement Plan) rather than a generic recommendation.

OP-1 — Workflow disruption during phased rollout of the new scheduling/check-in system disrupts ongoing patient care.

Likelihood	Impact	Severity
Medium	High	High

Follow the phased WBS rollout sequence (Scheduling & Notifications MVP → Resource Visibility & Records → Reporting & Handoff) rather than a single cutover. Require UAT sign-off from Administrative Staff, Doctors, and Nurses before each phase goes live. Schedule each go-live during lower-volume periods, avoiding the 6 PM peak identified in the dashboard analysis. Increase Hospital Leadership/IT engagement to daily stand-ups during each cutover week.

OP-2 — Clinical and administrative staff resist adopting new automated workflows, reverting to manual habits.

Likelihood	Impact	Severity
High	Medium	High

Schedule comprehensive, role-specific training sessions for each stakeholder group ahead of each rollout phase, per the "manage closely" engagement strategy already defined for Doctors, Nurses, and Administrative Staff. Designate

Likelihood	Impact	Severity
department champions to model adoption and answer peer questions. Hold weekly feedback sessions during rollout to surface and address concerns early.		

OP-4 — Resource shortages (nurses, rooms, equipment) during go-live create bottlenecks.

Likelihood	Impact	Severity
Medium	High	High
Deploy the FR3 real-time resource-visibility dashboard ahead of go-live so shortages are flagged proactively rather than discovered after the fact. Establish a cross-department resource-sharing protocol specifically for the 6 PM peak-demand window identified in the dashboard. Review resource-availability history (Unavailable/Under Maintenance patterns) by department before scheduling each phase's go-live date.		

TE-1 — A data breach or exploited security vulnerability compromises patient data.

Likelihood	Impact	Severity
Medium	High	High
Conduct a full security audit and penetration test of legacy and integrated systems before go-live (NFR3). Enforce encryption protocols for patient data in transit and at rest across the scheduling and records systems. Require formal security sign-off as a go/no-go gate at WBS Milestone M5, before any further rollout proceeds.		

TE-2 — System/network downtime disrupts scheduling, notifications, or dashboard availability.

Likelihood	Impact	Severity
High	Medium	High
Implement the NFR2 reliability requirements (redundant network paths, monitoring alerts) before go-live. Document and test a downtime contingency/failover plan with IT. Monitor uptime continuously as a KPI alongside the wait-time and satisfaction metrics already tracked on the dashboard.		

TE-3 — Integration between the scheduling module and patient records system (FR5) fails or creates duplicate/inconsistent records.

Likelihood	Impact	Severity
Medium	High	High
Sequence FR5 integration work with dedicated UAT (WBS Phase 6, Milestone M7) before Phase 2 go-live. Run parallel reconciliation checks between old and new systems throughout the transition period. Define and test data-validation rules (e.g., duplicate patient ID checks) before integration go-live.		

TE-4 — Legacy data-quality issues (missing IDs, "N/A" placeholders, single-key lookup limitations) mislead decision-making.

Likelihood	Impact	Severity
High	Medium	High

Apply the documented data-cleaning standards (ID validation, standardized missing-value handling) as an ongoing checkpoint for any new data feeding the dashboards. Label known lookup limitations directly on any dashboard or report that uses them, consistent with the practice already established in the Data Visualization Document. Periodically audit a sample of records against source systems to catch new data-quality issues early.

3. Contingency Plans

The contingency plans below define what to do if a High-priority risk occurs despite mitigation, with the goal of minimizing disruption and recovering timelines as quickly as possible.

Risk ID	Contingency Plan
OP-1	Activate rollback to the prior manual process for the affected workflow immediately and notify Hospital Leadership and affected departments within 1 hour. Stand up a temporary manual scheduling/check-in desk staffed by Administrative Staff to absorb patient flow while the issue is diagnosed. Conduct a root-cause review with IT and the Project Manager within 24 hours, and do not proceed to the next WBS phase until the issue is resolved and re-tested.
OP-2	If resistance persists past the first two weeks of a phase, escalate to Hospital Leadership for reinforced messaging and temporarily dual-run the old and new processes to reduce pressure. Provide additional one-on-one coaching and re-survey satisfaction with the new workflow after two more weeks. If adoption still does not improve, use the Scope Change Management Process to reassess the rollout timeline for that department.
OP-4	Activate the cross-department resource-sharing protocol immediately and reprioritize non-urgent resource-dependent appointments. Notify affected patients proactively of any resulting delay via the FR2 notification feature or a manual fallback. Log the shortage event and review it at the next steering committee meeting to determine if a permanent resource reallocation is needed.
TE-1	Immediately isolate affected systems from the network and notify the IT Manager and Hospital Leadership within 1 hour. Conduct a root-cause analysis and apply emergency fixes/patches before restoring system access. Notify affected patients and relevant regulatory bodies per applicable data-protection requirements (e.g., HIPAA or the local equivalent), and document the incident for the compliance record.
TE-2	Switch immediately to the documented manual fallback process (phone/paper scheduling and check-in) until systems are restored. IT provides hourly status updates to Administrative Staff and Hospital Leadership until resolution. After restoration, reconcile any manually recorded appointments/check-ins back into the system to prevent data loss.
TE-3	Revert to the pre-integration records process immediately and freeze new record creation in the failed system until resolved. Run a data-reconciliation script to identify and correct duplicate/inconsistent records created during the failure window. Delay the next WBS milestone until IT confirms the integration defect is fixed and re-tested.
TE-4	Immediately flag the affected report to Hospital Leadership and pause any decision still pending on it. Re-run the analysis with corrected data and reissue the report with a documented correction note. Review the data-pipeline step that allowed the issue through and add a validation check to prevent recurrence.

4. Updated Risk Matrix

The table below consolidates the High-priority Risk Matrix with its Mitigation Strategy column, as required for this lab. The full 15-risk matrix (including Medium/Low risks) and the contingency plans in table form are provided in the companion workbook Risk_Matrix_Mitigation.xlsx (sheets: High_Priority_Risk_Matrix, Contingency_Plans, and Full_Risk_Matrix for reference).

Risk ID	Risk Description	Likelihood	Impact	Severity	Mitigation Strategy
OP-1	Workflow disruption during phased rollout of the new scheduling/check-in system disrupts ongoing patient care.	Medium	High	High	Follow the phased WBS rollout sequence (Scheduling & Notifications MVP → Resource Visibility & Records → Reporting & Handoff) rather than a single cutover. Require UAT sign-off from Administrative Staff, Doctors, and Nurses before each phase goes live. Schedule each go-live during lower-volume periods, avoiding the 6 PM peak identified in the dashboard analysis. Increase Hospital Leadership/IT engagement to daily stand-ups during each cutover week.
OP-2	Clinical and administrative staff resist adopting new automated workflows, reverting to manual habits.	High	Medium	High	Schedule comprehensive, role-specific training sessions for each stakeholder group ahead of each rollout phase, per the "manage closely" engagement strategy already defined for Doctors, Nurses, and Administrative Staff. Designate department champions to model adoption and answer peer questions. Hold weekly feedback sessions during rollout to surface and address concerns early.
OP-4	Resource shortages (nurses, rooms, equipment) during go-live create bottlenecks.	Medium	High	High	Deploy the FR3 real-time resource-visibility dashboard ahead of go-live so shortages are flagged proactively rather than discovered after the fact. Establish a cross-department resource-sharing protocol specifically for the 6 PM peak-demand window identified in the dashboard. Review resource-availability history (Unavailable/Under Maintenance patterns) by department before scheduling each phase's go-live date.
TE-1	A data breach or exploited security vulnerability compromises patient data.	Medium	High	High	Conduct a full security audit and penetration test of legacy and integrated systems before go-live (NFR3). Enforce encryption protocols for patient data in transit and at rest across the scheduling and records systems. Require formal security sign-off as a go/no-go gate at WBS Milestone M5, before any further rollout proceeds.
TE-2	System/network downtime disrupts scheduling, notifications, or dashboard availability.	High	Medium	High	Implement the NFR2 reliability requirements (redundant network paths, monitoring alerts) before go-live. Document and test a downtime contingency/failover plan with IT. Monitor uptime continuously as a KPI alongside the wait-time and satisfaction metrics already tracked on the dashboard.
TE-3	Integration between the scheduling module and patient records system (FR5) fails or creates duplicate/inconsistent records.	Medium	High	High	Sequence FR5 integration work with dedicated UAT (WBS Phase 6, Milestone M7) before Phase 2 go-live. Run parallel reconciliation checks between old and new systems throughout the transition period. Define and test data-validation rules (e.g., duplicate patient ID checks) before integration go-live.
TE-4	Legacy data-quality issues (missing IDs, "N/A" placeholders, single-key lookup	High	Medium	High	Apply the documented data-cleaning standards (ID validation, standardized missing-value handling) as an ongoing checkpoint for any new data feeding the dashboards. Label known lookup limitations directly

	limitations) mislead decision-making.				on any dashboard or report that uses them, consistent with the practice already established in the Data Visualization Document. Periodically audit a sample of records against source systems to catch new data-quality issues early.
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5. Summary					
• All seven High-priority risks now have both a proactive mitigation strategy and a reactive contingency plan, closing the gap left by the prior lab's register (which named mitigations but not contingencies). • Every contingency plan assigns a first responder (IT Manager, Project Manager, Administrative Staff, or Hospital Leadership) and a first action, so the response does not depend on improvisation in the moment. • Three risks (TE-1, TE-2, TE-3) gate specifically on WBS milestones (M5, M7) — their mitigations are therefore go/no-go criteria, not background activities, and should be tracked accordingly in the project schedule. <i>This Risk Mitigation Plan should be reviewed and updated at each WBS phase gate, and immediately after any contingency plan is invoked, to capture lessons learned before the next phase begins.</i>					